YOUR PROGRAMME

The Calm Nights *Method*

A structured, self-guided programme for helping your child learn to fall asleep — and fall back asleep — without cry-it-out. Six steps, in order. Read steps 1–4 before night one; then come back to the step you're on.

STEP 1 · UNDERSTAND

How sleep works

Hello, and welcome. I'm Georgia — a certified pediatric sleep coach and a parent too, so I know how heavy those 3am wake-ups feel. Calm Nights brings together the general principles I teach every family, set out as a clear path you can follow on your own.

> **What this is — and isn't.** Calm Nights is general education and a general implementation framework. It isn't a plan written for your child: I haven't seen your child's history, and it doesn't cover naps or daily schedules. If you'd like that, my 1:1 packages are built for it. Everything here works best for healthy children from 6 months to around 4 years whose main challenge is falling and staying asleep.

Four ideas the whole programme rests on

- **Sleep pressure.** The longer we're awake, the stronger the body's drive to sleep. A consistent wake-up time and a well-timed bedtime mean that drive peaks when you need it to.
- **Sleep cycles.** Every 45–90 minutes we all surface briefly between cycles. Adults roll over. Children who fell asleep being fed, rocked or held often look for that same help again — and that's the wake-up.

- **Sleep associations.** Whatever your child needs to *fall* asleep is usually what they'll need to fall *back* asleep. Change how bedtime ends, and nights follow.
- **Predictability feels safe.** Children settle when they know what happens next. The same routine, the same words and the same response — every time — matter more than any single technique.

> **The promise of this programme:** consistency over perfection. Clear, repeated cues. Calm, predictable responses from you. *Comfort is never removed; confusion is.*

Understanding crying

When something familiar changes, most children protest. That's communication, not harm — imagine someone suddenly took away your favourite pillow. I don't use or teach cry-it-out: you stay present and responsive throughout. Once you know their needs are met (fed, clean, comfortable, well), use this scale:

1	2	3	4	5	6	7	8	9	10

1–5 · Grizzling, whining, on-off fussing: they're working it out — pause and let them. **6–7 · Building, sustained crying:** respond with your starting point. **8–10 · Distressed, frantic, or a different "something's wrong" cry:** go in, check, comfort, then restart calmly.

Why it's worth it

Sleep is when growth hormone is released, memories are consolidated and little bodies repair. Good sleep habits are linked with better mood, attention, vocabulary, memory and resilience. Helping your child learn to fall asleep on their own is one of the most loving, long-lasting gifts you can give them — and yourselves.

STEP 2 · CHECK FIT & READINESS

Is this right for us, right now?

Calm Nights is a good fit when…

- ✓ Your child is 6 months to around 4 years, healthy and growing well.
- ✓ They usually need a feed, rocking, being held or you lying beside them to fall asleep — and wake looking for it.
- ✓ You're ready to follow one approach consistently for 14 nights, without day-to-day support.

Get personalised support first if…

- ☐ Your baby is under 6 months (or under 6 months adjusted age if born early)
- ☐ Reflux, allergies, snoring, pauses in breathing, weight concerns or ongoing pain
- ☐ Neurodivergence, additional needs or developmental delay
- ☐ Twins or multiples, or siblings sharing a room
- ☐ Naps and the daily schedule are the main problem

Any of these means a general framework is likely to frustrate you. That's not a judgement — it's why my 1:1 packages and Specialised Sleep Support exist.

Readiness check

Starting before you're ready is the number one reason families give up on night three. Tick through these honestly.

Your child

- ☐ Healthy right now — no illness, ear infection or new teeth cutting through
- ☐ Your pediatrician is happy with growth and with any changes to night feeds
- ☐ No big change in the next two weeks (travel, new sibling, starting daycare)

Your family

- ☐ Everyone who does bedtime has read steps 1–4
- ☐ You have two weeks with mostly home bedtimes
- ☐ You've planned something kind for yourselves (early nights, help with the older kids)

If several boxes are unticked, that's information, not failure. Use the free Gentle Sleep Reset for a few days to stabilise, then begin.

Safety

This programme isn't medical advice. Please check with your pediatrician before you start, pause if your child becomes unwell, and always follow safe sleep guidance: alone, on their back, in a crib with nothing but a fitted sheet (the free Sleep Tight checklist covers everything).

If you ever feel overwhelmed: lay your child down safely in their crib, step out, and give yourself a few minutes — a shower, music, a cup of tea, a call to a friend. Even the most loving, exhausted parent can reach breaking point; stepping away is a safe and responsible choice. If nothing soothes your child, contact your doctor. And if you've been feeling low or anxious for a while, please tell your own doctor too.

STEP 3 · PREPARE

Prep week

Do these in the 3–7 days before night one. Many families see improvement from preparation alone. The workbook has a tick-list version.

The room

- **Dark:** blackout curtains or window film. If you use a night light, choose a very dim, warm one, or move it to the hallway.
- **Calm:** remove mobiles and stimulating décor from the crib area.
- **Comfortable:** season-appropriate PJs and a sleep sack.
- **Sound:** white noise is fine — steady, all night, not too loud.

The feed

- Move the bedtime milk to the **start** of the routine — lights on, ideally outside the bedroom.
- Keep them awake and engaged for the whole feed.
- Follow it with bath or books so the last thing before sleep isn't sucking.
- Ask your pediatrician which night feeds (if any) they want you to keep.

The day

- Fix a morning wake-up time (within 30 minutes) and keep it, even after bad nights.
- Lots of daylight, movement and floor play — and practise new skills like standing and sitting down.
- Fill their cup: plenty of connected, phone-free time in the evening.

The team

- Choose your key phrase (below) — every adult uses the exact same words.
- Decide who does which nights. Tag-teaming helps.
- Agree now how you'll handle a hard night, so you don't debate it at 2am.
- From 12 months, tell your child what's changing with a simple bedtime story about them.

CHOOSE YOUR KEY PHRASE

- "It's sleepy time now. I love you very much. See you in the morning."
- "Night night, my love. Time for sleep. I'll see you when the sun comes up."

- “You’re safe, you’re loved, and it’s time to rest. Goodnight.”

Short, warm, identical every time. Repetition is what makes it reassuring — it tells your child exactly what happens next.

STEP 4 · CHOOSE

Choosing a Gentle Starting Point

There are two starting points. Both build the same skill — falling asleep without the old support — and both keep you present and responsive. The only difference is *how much of you is in the room*.

STARTING POINT A

Stay Close

Also called Cotside Comforting. You stay beside the crib with touch and voice, then move a little further away every few nights until you're out of the room.

STARTING POINT B

Step Out & Check

Also called Quick Checks. Key phrase, then you leave, returning for brief, calm, identical checks if crying builds.

Three questions

You don't need a perfect match. You need an approach you can repeat, the same way, every night.

1 **When you stay in the room at bedtime, does your child generally calm down or wind up?**

- Calms down with you nearby → **Stay Close**
- Gets more worked up, or plays and chats → **Step Out & Check**
- Not sure → **Stay Close**

2 **Which could you repeat at 2am, every night for two weeks?**

- Sitting quietly beside the crib through some protest → **Stay Close**
- Stepping out and returning for short checks → **Step Out & Check**

3 **Can every adult who does bedtime do it the same way?**

- Yes → keep your choice.

- No → choose the one everyone can do. Consistency between adults matters more than fit.

> **If your answers point different ways, question 2 wins.** The approach you can follow consistently will outperform the “right” one you can’t. Still unsure? Start with Stay Close — it’s the gentlest, and easy to move on from.

Then commit

- [ ] Write your choice and key phrase in the workbook.
- [ ] Give it 7 full nights before considering a switch. Night 7 is the only switch point, and you switch once.
- [ ] If neither starting point feels possible for your family, that’s a sign to get personalised support rather than force a fit.

In 1:1 coaching, I choose the approach with you after reviewing your child’s history, temperament and current sleep supports. This section is a general guide, not a personal recommendation.

STEP 5 · IMPLEMENT

The 14 nights

The bedtime routine

20–40 minutes. Same steps, same order, same calm energy, every night. Not long — predictable.

1

Feed (lights on)

Milk at the start, outside the bedroom where possible. Fully awake.

2

Bath or wash

Lukewarm, a gentle soak — a strong wind-down cue.

3

PJs, sleep sack, teeth

Into the dim bedroom from here on. Screens off.

4

Books & song

Two books, one song. Dim light. Lots of cuddles.

5

Key phrase

Into the crib calm and awake. Say your phrase.

6

Your starting point

Stay Close or Step Out & Check — the same every time.

Time it well. Overtired children fight sleep. For most babies and toddlers, lights out between 6:30 and 8:00pm works well. If bedtime has crept later, bring it earlier by 15 minutes every couple of nights.

Down awake. Lay your child down calm but awake — drowsy is fine, asleep is not. Falling asleep in the place they'll wake up is what lets them reconnect sleep cycles alone at 2am.

STARTING POINT A

Stay Close

You stay with your child as they learn to fall asleep, then reduce your presence every few nights until you're out of the room. Your child is never alone while learning; they just need you a little less each time.

Nights 1–3

BESIDE THE CRIB

Sit or kneel right next to the crib. Offer touch — a hand on the tummy, stroking the back or head — and soft shushing. Reduce touch as they calm; stop before they're asleep if you can. No picking up.

Nights 4–6

BESIDE THE CRIB, VOICE ONLY

Same spot, but hands off. Comfort with your voice and presence. If crying builds past a 6 out of 10, a brief touch is fine — then back to voice.

Nights 7–9

HALFWAY TO THE DOOR

Move your chair halfway across the room. Calm, low, repetitive reassurance (your key phrase, a gentle "shhh"). Keep eye contact soft and minimal.

Nights 10–12

IN THE DOORWAY

Sit in the open doorway where they can see or hear you. Reassure with your key phrase at intervals rather than constantly.

Nights 13–14

JUST OUTSIDE THE DOOR

Out of sight, within earshot. Offer your key phrase from outside if needed.

- **Move on schedule**, even if the previous stage was going well. Lingering is how your presence becomes the new support.
- **No picking up or rocking** — it sends a mixed message. If they stand, gently lay them down once, then let them sit or stand while you stay calm.
- **Boring is your friend:** low voice, soft eyes, minimal chat.

- **Wake-ups get the same response** as bedtime, from the stage you're at that night.

STARTING POINT B

Step Out & Check

You leave after your key phrase and return for brief, calm checks if needed — loving reassurance with a clear, consistent boundary.

THE SCRIPT

1. **Key phrase & leave.** Crib, calm-but-awake, your key phrase, and walk out with warmth and confidence.
2. **Pause — at least 5 minutes.** Check the clock (five minutes at bedtime feels like twenty). If crying is steady or settling, keep pausing.
3. **Check briefly.** If crying is building (6+), go in. Make sure all is well. A quick stroke on the tummy, back or head. No picking up, no lying down with them.
4. **Key phrase & leave again.** Calm, loving, firm. Under a minute in the room.
5. **Repeat** until they're asleep, keeping pauses at least as long as the last one.

Many children fall asleep around the 15–20 minute mark. If you're comfortable, letting pauses stretch towards that gives your child real space to practise. If it feels too long, keep pauses shorter but *consistent*. If checks seem to make crying louder, lengthen the pause — and keep every check identical: same words, same touch, same exit.

Wake-ups & early mornings

Pause before you respond. Many night noises aren't full wake-ups — they're your child moving between sleep cycles. Wait around 5 minutes; then, if needed, respond with **exactly the same starting point** you used at bedtime.

Night feeds. If your pediatrician wants you to keep a night feed, keep it — calmly, lights low, then straight back into the crib with your key phrase. For comfort feeds your doctor is happy to drop, reduce gradually: a couple of minutes off a breastfeed, or around 30ml (1oz) off a bottle, every night or two.

The 6am rule. Anything before 6am is treated as night: same pause, same response. Between 6 and 7am, if they're content, you can leave them until a more family-friendly hour. If they're still

asleep 12–12.5 hours after bedtime, gently wake them.

The boring morning. Open the curtains with a cheerful “good morning”. Do the boring things first — diaper, dressed, teeth — then offer the first milk *outside* the bedroom. Then the cuddles. A toddler clock is a great help from about two.

STEP 6 · REVIEW

Review checkpoints

Progress hides on hard nights. Three short checkpoints — with your tracker in front of you — tell you what to do next. You'll also get a check-in email at each one.

Night 4 · Is it moving?

Look for:

- Lay-down is a little calmer
- Falling asleep takes less time than night 1
- At least one wake-up was shorter or quieter

KEEP GOING

Any one of these is progress. Keep going exactly as you are — no changes.

ADJUST

None yet, or night 3 felt worse? That's common. Run the "Check the basics" list in Troubleshooting, change nothing else, and hold steady to night 7.

GET SUPPORT

Crying is escalating to distress (8–10) for long stretches every night, or your child seems unwell: pause, speak to your pediatrician, and consider personalised support.

Night 7 · Is the skill landing?

Look for:

- Most bedtimes, they fall asleep with your starting point, not the old support
- Fewer wake-ups, or shorter ones
- You feel clearer about what to do at 2am

KEEP GOING

Keep going to night 14. If you're on Stay Close, keep moving on schedule.

ADJUST

Bedtime is better but nights aren't: check every wake-up gets the same response as bedtime. Use the Troubleshooting library for the specific sticking point.

GET SUPPORT

Little or no change despite following the plan every time (including 3am)? That's not a failure — it's information that your child needs a plan built around them.

Night 14 · Where have we landed?

Look for:

- They fall asleep on their own most nights
- Wake-ups resettle with a brief, calm response — or don't happen
- Mornings start at 6am or later

KEEP GOING

Wonderful. Move to "Keeping it going" in the plan.

ADJUST

Mostly there? Hold the same plan for another 7 nights before changing anything. Progress often finishes quietly.

GET SUPPORT

Still waking frequently, splitting nights, early rising before 5:30am, or nap problems undoing nights? This is where personalised assessment adds what a general plan can't.

STEP 6 · TROUBLESHOOT

Troubleshooting

General answers to the most common sticking points. Change one thing at a time, and give any change three nights.

▼ Check the basics (do this first)

Is the milk feed truly at the start of the routine, with books or bath after it? Is bedtime well-timed (not overtired)? Is the morning wake-up time consistent? Is every adult using the same starting point and key phrase — including at 3am? Most stalls come from one of these.

▼ “Night 3 got worse!”

Very common, and usually a sign your child is testing whether the new pattern is real. Hold steady to your night 4 checkpoint. Families who change course here start again from zero.

▼ Illness or teething

Pause, comfort as much as they need, and follow medical advice. Keep the routine and wake-up time. When they’re well (usually 2–3 days after symptoms ease), restart at the stage you’d reached — it comes back quickly.

▼ They stand up and won’t lie down

Practise “up and down” lots during the day. At night, lay them down once, then leave them to it — they will lie down when tired. Repeatedly laying them down becomes a game.

▼ Vomiting from crying

Clean up calmly and quickly with low light and little interaction, change the sheet, then continue your starting point. If it happens repeatedly, pause and speak to your pediatrician.

▼ Early rising (before 6am)

Treat it as night: same pause, same response. Then look at the day: is bedtime too late? Is the room light at dawn? Is there an early-morning feed still being offered?

▼ Travel & time zones

Bring the routine, sleep sack, white noise and key phrase. Shift to local time straight away with daylight and mealtimes. Expect a few unsettled nights, then return to your starting point.

▼ The toddler negotiator

Use the bedtime chart and Bedtime Pass cards. Offer choices before bed (“blue PJs or stars?”), not after. Once in bed: key phrase, same response, no new debates.

▼ We disagree as parents

Agree the starting point and the hard-night plan before night one. If one parent finds settling harder, they take a different role (bath and books) while the other leads. Unity matters more than which starting point you chose.

▼ Our checkpoint says “get support”

Then that’s the right next step, not a failure. See “Keeping it going” below — Georgia will build on what you’ve already done.

YOUR TOOLS

Workbook & tracker

The printable workbook (27 pages) is where you make the method yours and see your progress. It follows the same six steps:

- **Steps 1–2 · Before you begin** — sleep basics and the crying scale, your child's current sleep associations, the fit and readiness check, and a three-night baseline log.
- **Steps 3–4 · Prepare & choose** — a day-by-day prep-week planner, the three-question starting-point worksheet, your plan at a glance, routine builder, key phrase, and team and hard-night plan.
- **Step 5 · The 14 nights** — the Stay Close stage map, a Step Out & Check log, a wake-ups and feed-reduction page, a night-by-night journal and the 14-night tracker.
- **Step 6 · Review** — one page each for your night 4, 7 and 14 checkpoints, the troubleshooting library with a change log, life after night 14, and cut-out script cards.

AFTER NIGHT 14

Keeping it going

Sleep isn't about perfection — it's about consistency, intention and loving boundaries. There will be wobbles: a cold, a new tooth, a holiday. When they come, steady the foundations with the free Gentle Sleep Reset, then pick your starting point back up. It returns faster every time.

If sleep is calm but one night-time habit lingers — a single feed, say — From Reset to Rest is my protocol for reducing one support gradually.

You are doing something truly wonderful for your child — and for yourselves. You've got this.

— Georgia

▶ What's in Calm Nights, and what isn't

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